

1. Information about the indicator	
The goal	Ensure healthy lives and promote well-being for all at all ages
Task	By 2030, end the epidemics of AIDS, tuberculosis, malaria and neglected tropical diseases and combat hepatitis, water-borne diseases and other communicable diseases
Indicator name of the global indicator National indicator	Tuberculosis incidence per 100,000 population
2. Information about the organization	
Organization	Ministry of Health of the Republic of Kazakhstan
3. Definitions and concepts	
Definition	The tuberculosis incidence rate per 100,000 people is defined as the estimated number of new or newly acquired tuberculosis cases (all forms of tuberculosis, including people with AIDS, are taken into account) that occurred in a given year, per 100,000 population.
Basic concepts	Direct measurement requires high-quality surveillance systems, which have a low level of underestimation, and a high-quality healthcare system, so that the probability of underestimation of the diagnosis is also minimal. Otherwise, indirect estimates will be based on registration data and estimates of the level of underestimation and underestimation of the diagnosis.
Justification and explanation	After two years of consultations, the World Health Assembly approved a new Global Strategy to Combat Tuberculosis after 2015 in May 2015. Known as the Final Tuberculosis Strategy, it covers the period 2016-2035. The overall goal of the strategy is to "End the global tuberculosis epidemic", and accordingly, in the context of the SDGs, ambitious targets have been set to reduce tuberculosis cases and deaths for 2030 (80% reduction in morbidity compared to 2015) and for 2035 (90% reduction in morbidity). The Tuberculosis Incidence Rate indicator was chosen to measure the reduction in the number of tuberculosis cases. Despite the fact that this indicator in most countries was estimated in 2014 with a high level of uncertainty, national authorities provide a good proxy indicator (substitute) for cases where there is a limited underestimation of detected cases or limited underestimation or overestimation of cases of diagnosis.
4. Sources and collection methods	
Data sources	Administrative data
Data collection methods	The data comes from an administrative source and is available on an annual basis.
Unit of measurement	le Per 100,000 peop
Frequency	Monthly
5. Calculation method and other methodological considerations	
Calculation method:	Tuberculosis incidence estimates are made as part of a consultative and analytical process conducted by WHO and are published annually. These estimates are based on annual case notifications, an assessment of the quality and coverage of tuberculosis incidence data, national tuberculosis prevalence surveys, and information from population movement

	accounting systems. Morbidity estimates for each country are made using one or more of the following approaches, depending on the available data: (i) Incidence rate = number of case notifications/ estimated proportion of cases detected; (ii) the double coverage method (iii) incidence rate = prevalence of the disease/ duration of the condition. The uncertainty bounds are provided in addition to the best estimates.
Reviews and restrictions:	The tuberculosis incidence rate has been used for a century as the main indicator of the tuberculosis burden, together with the tuberculosis mortality rate. This indicator allows for comparisons over time and between countries. Improving the quality of TB surveillance will reduce uncertainty about the values of the indicator.
6. Data availability and fragmentation	
Data availability and gaps:	The data is available on the website of the Bureau of National Statistics of the Agency for Strategic Planning and Reforms of the Republic of Kazakhstan.
Disassembly level:	The indicator is disaggregated by gender and age (children and adults), by place of residence (urban/rural), and geographical regions.
7. Comparison with international data / standards	The population used as the denominator in calculations from national sources may differ from data from the United Nations Population Division (UNPD). WHO uses population estimates from the United Nations Population Division (UNPD).
8. References and documentation	-----